

DYNA TICLOPIDINE TABLET 250MG

MAL08111762AZ

EACH TABLET CONTAINS:

Ticlopidine Hydrochloride 250 mg

DESCRIPTION:

TABLET

Colour : Pink
Shape : Round and Biconvex
Coating : Film-coated

PHARMACODYNAMICS:

Ticlopidine is an inhibitor of platelet aggregation. It causes dose-dependent inhibition of platelet aggregation and release of platelet factors, as well as a prolongation of bleeding time. The drug has no significant *in vitro* activity, but only *in vivo* activity; however, there is no evidence for a circulating active metabolite.

PHARMACOKINETICS:

After oral administration of a single standard dose of Ticlopidine, a rapid absorption occurs, with peak plasma levels occurring approximately 2 hours after dosing.

Absorption is practically complete. Administration of Ticlopidine after meals improves bioavailability.

Steady-state plasma levels are obtained after 7-10 days of dosing at 250 mg twice daily. The average terminal elimination half-life of Ticlopidine at steady state is approximately 30-50 hours. However, the inhibition of platelet aggregation does not correlate with plasma drug levels.

Ticlopidine is extensively metabolized by the liver. Following an oral dose of radiolabeled product, 50-60% are recovered in the urine and 20-30% in faeces.

INDICATION:

Maintenance of patency of coronary artery by-pass graft.

Maintenance of patency of access sites in patients on chronic haemodialysis.

Reducing risk of thrombotic stroke (fatal or nonfatal) in patients who have experienced stroke precursors and in patients who have had a completed stroke.

Prevention of subacute occlusions following coronary STENT(s) implantation.

Because Ticlopidine hydrochloride is associated with a risk of neutropenia/granulocytosis, which may be life-threatening (see Adverse Reactions), Ticlopidine hydrochloride should be reserved for patients who are intolerant to aspirin therapy where indicated to prevent stroke.

RECOMMENDED DOSE:

The tablets should be taken with meals.

Adults: Usual Dosage: 2 tablets daily, with meals.

In case of prevention of subacute occlusions following coronary STENT(s) implantation, treatment may be started just before or just after STENT implantation and should be continued for about 1 month (2 tablets/day) combined with aspirin (100-325 mg/day).

Children: Not indicated.

Elderly: The major clinical trials were conducted in an elderly population with a mean age of 64 years.

Although Ticlopidine pharmacokinetics are modified in elderly subjects, the pharmacological and therapeutic activity at a dose of 500 mg/day are unaffected by age.

ROUTE OF ADMINISTRATION:

FOR ORAL USE ONLY

CONTRAINDICATION:

Haemorrhagic diathesis.

Organic lesions which are liable to bleed; active gastroduodenal ulcer or haemorrhagic cerebrovascular accident in the acute phase.

Blood disease involving prolonged bleeding time.

History of allergy to Ticlopidine.

History of leukopenia, thrombocytopenia and agranulocytosis.

WARNING AND PRECAUTIONS:

Precautions: Haematological Monitoring: CBCs with differential and platelet counts should be performed at the start of treatment and then every 2 weeks for the first 3 months of therapy and within 15 days after discontinuation of Ticlopidine, should the treatment be stopped within the first 3 months of therapy.

In the event of neutropenia (<1500 neutrophils/mm³) or thrombocytopenia ($<100,000$ platelets/mm³), treatment should be discontinued and CBCs, with differential and platelet counts, should be monitored until they return to normal.

Clinical Monitoring: All patients should be carefully monitored for clinical signs and symptoms of adverse drug reactions especially during the first 3 months of therapy.

The signs and symptoms possibly related to neutropenia (fever, sore throat, ulcerations in oral cavity), thrombocytopenia and/or abnormal haemostasis (prolonged or unusual bleeding, bruising, purpura, dark stool), and jaundice (including dark urine, light-coloured stool) should be explained to the patient.

All patients should be advised to stop medication and consult their physician immediately if any of the above signs or symptoms occur.

The decision to restart treatment should only be taken according to clinical and laboratory findings.

The clinical diagnosis of thrombotic thrombocytopenic purpura (TTP) is characterized by the presence of thrombocytopenia, haemolytic anemia, neurological symptoms, renal dysfunction and fever.

The onset may occur suddenly. Most cases were reported in the first 8 weeks of initiating therapy.

Due to the risk of fatal outcome, in the event of suspected thrombotic thrombocytopenic purpura, a specialist team should be contacted.

Treatment with plasmapheresis has been reported to improve the prognosis.

Haemostasis: Ticlopidine must be used with caution in patients who are at increased risk of bleeding.

The drug should not be given in combination with heparins, oral anticoagulants and antiplatelet drugs (see Warnings and Interactions); however, in exceptional cases of concomitant treatment, close clinical and laboratory monitoring is required, and should include bleeding time.

If the patient is to undergo elective surgery, treatment should, wherever possible, be stopped at least 10 days before surgery.

In the event of emergency surgery, 3 means may be used alone or in combination to limit the risk of haemorrhage and prolonged bleeding time:

Administration of 0.5-1 mg/kg of methylprednisolone IV, to be renewed; desmopressin at a dosage of 0.2-0.4 mcg/kg; platelet transfusions.

As Ticlopidine is extensively metabolized by the liver, the drug should be used with caution in patients with impaired liver function, and treatment should be discontinued if hepatitis or jaundice develops.

Warning: Haematological and haemorrhagic adverse effects can occur. These may be severe and sometimes fatal consequences have been observed (see Adverse Reactions).

These severe effects could be associated with: Inadequate monitoring; late diagnosis and inappropriate therapeutic measures for adverse effects; concomitant administration of anticoagulants or antiplatelet agents eg, aspirin and NSAIDs. However, in case of STENT implantation, Ticlopidine should be combined with aspirin (100-325 mg/day) for about 1 month following implantation.

DRUG INTERACTIONS:

Combinations With Increased Haemorrhagic Risk: NSAIDs: Increase of haemorrhagic risk (increase of platelet antiaggregant activity and NSAIDs effect on the gastroduodenal mucous membrane). If such drugs are necessary, close clinical monitoring is required.

Antiplatelet Drugs: Increase of haemorrhagic risks (increase of platelet antiaggregant activity). If such drugs are necessary, close clinical monitoring is required.

Salicylic Derivatives (by extrapolation from acetylsalicylic acid): Increase of haemorrhagic risk (increase of platelet antiaggregant activity and salicylic derivatives effect on the gastroduodenal mucous membrane). If such drugs are necessary, close clinical monitoring is required.

Combinations Requiring Precautions for Use: Theophylline (base and salts) and Aminophylline: Increase of Theophylline blood level with the risk of overdose (due to decreased plasma clearance of Theophylline).

Clinical monitoring, as well as possible monitoring of Theophylline blood levels. If necessary, Theophylline dosage should be adjusted during treatment with Ticlopidine and after its discontinuation.

Phenytoin, Phosphenytoin: Increase in of Phenytoin plasma concentration with symptoms of overdose (due to inhibition of Phenytoin metabolism). Clinical monitoring as well as monitoring of plasma concentrations should be performed.

Ciclosporin: Decrease of Ciclosporin blood concentration. Ciclosporin dosage should be increased while monitoring blood concentrations. Reduce the dosage if Ticlopidine is discontinued.

Incompatibilities: None.

PREGNANCY AND LACTATION:

The safety of Ticlopidine in pregnant or nursing women has not been established. Studies in rats have shown that Ticlopidine is excreted in the milk. Unless absolutely indicated, Ticlopidine should not be prescribed to pregnant or nursing women.

SIDE EFFECTS/ADVERSE REACTIONS:

The incidence rates of adverse effects are derived from 2 multi-center controlled clinical trials (CATS and TASS) conducted in 2048 TIA/stroke patients.

Haematological: Cell blood counts were closely monitored in 2 large clinical studies conducted in 2048 TIA/stroke patients treated with Ticlopidine. A 2.4% incidence of neutropenia was reported, which includes 0.8% of severe neutropenia (<450 neutrophils/mm³).

In these clinical trials, as in most of the cases from the post-marketing surveillance studies, most cases of severe neutropenia or agranulocytosis (<300 neutrophils/mm³) developed in the first 3 months of Ticlopidine treatment, and were not typically accompanied by signs of infection or other clinical symptoms (need for CBCs monitoring). In such cases, the bone marrow typically showed a decrease in myeloid precursors.

There have been rare reports of bone marrow aplasia or pancytopenia.

Other Reported Haematologic Disorders: Thrombocytopaenia, either isolated or associated with haemolytic anaemia. TTP with thrombocytopaenia, haemolytic anaemia, neurological disorders, renal impairment and fever (see Warnings and Precautions).

Haemorrhagic: Common haemorrhagic complications, mainly bruising or ecchymosis and epistaxis may occur with treatment. Peri- and postoperative bleedings have been reported (see Precautions).

Gastrointestinal: Treatment with Ticlopidine may cause gastrointestinal disturbances. The most common was diarrhoea and second was nausea. Diarrhoea is generally mild and transient, and occurs mainly during the first 3 months of treatment.

These disturbances usually resolve within 1-2 weeks without discontinuation of treatment. Very rare cases of severe diarrhoea with colitis have been reported. If the effect is severe and persistent, therapy should be discontinued.

Cutaneous: Common skin rashes, particularly maculopapular or urticarial, often accompanied by pruritus, have been reported. In general, skin rashes occur within the first 3 months of treatment with a mean time to onset of 11 days. If treatment is discontinued, the symptoms disappear within a few days. These skin rashes may be generalized.

Hepatic: There have been rare reports of hepatitis and cholestatic jaundice during the first months of treatment. The course has generally been favourable after treatment was discontinued.

Immunological Reactions: Very rare cases of immunological reactions with different manifestations have been reported eg, Quincke edema, vasculitis, lupus syndrome, hypersensitivity and nephropathy.

Altered Laboratory Findings: Haematological: Neutropenia and rarely, pancytopenia as well as isolated thrombocytopenia or exceptionally accompanied by haemolytic anaemia, have been associated with Ticlopidine treatment.

Hepatic: Ticlopidine treatment has been accompanied by an increase in hepatic enzymes. Common increase (either isolated or not), of alkaline phosphatases and transaminases (incidence greater than twice the upper limit of normal) was observed in both group (Ticlopidine and placebo). Ticlopidine treatment has also been accompanied by a minor elevation of bilirubin.

Cholesterol: Chronic Ticlopidine therapy has been associated with increased serum cholesterol and triglyceride levels. Serum HDL-C, LDL-C, VLDL-C and triglyceride levels may increase 8-10% after 1-4 months of treatment. No further elevations are seen with continued therapy. The ratios of the lipoprotein subfractions (especially the ratio of HDL to LDL) remain unchanged. The data in the clinical studies have shown that the effect does not depend on age, sex, alcohol consumption or diabetes, and has no influence on the cardiovascular risk.

SYMPTOMS AND TREATMENT OF OVERDOSE:

Based on animal studies, overdosage may cause severe gastrointestinal intolerance. Careful monitoring of vital signs and haemostasis (bleeding times) is necessary. In case of overdosage, it is recommended to begin gastric lavage and to apply standard supportive measures.

PACKING/PACK SIZE(S):

Blister pack of 3x10's, 4x10's and 10x10's.

JAUHI DARIPADA KANAK-KANAK

KEEP OUT OF REACH OF CHILDREN

MANUFACTURER/PRODUCT REGISTRATION HOLDER:

DYNAPHARM (M) SDN. BHD. 198001011897 (65683-V)

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